CHAPTER 2

The Asylum Era

The rise of the asylum is the story of good intentions gone bad. That the dreams of the early psychiatrists failed is unquestionable. By World War I, asylums had become vast warehouses for the chronically insane and demented. Yet whether the failure of the asylum lay in the nature of the enterprise itself is a matter of controversy. Some argue that the asylum failed because it was overwhelmed by the ever-rising numbers of psychiatric patients in the nineteenth century. Others maintain that many people admitted to the asylum had no psychiatric illness and were confined merely because they were social misfits and outcasts, inconvenient rather than ill. Through this historical debate, as through psychiatry itself, runs the cleft demarcating neuroscience from psychosocial understanding. The neuroscientific side of the story sees growing pathology; the psychosocial version sees a social universe increasingly intolerant of deviance.

This debate is one of the most exciting in the social history of medicine. At the very beginning, therefore, I want to make my own point of view clear. I side with the people who see the increasing numbers of psychiatric patients as the result of changing patterns of psychiatric disease. But there is a psychosocial component in addition. It concerns the redistribution of people who were already mentally ill to asylums. “Already mentally ill” must be emphasized, for some scholars argue that those being redistributed from family and almshouse to the asylum were merely deviant or intolerable, and not ill at all.

At the end of the story, the asylum fails. But this does not represent a failure of the biological paradigm as a model for diagnosing and treating patients. It represents the tragedy of individuals whose good therapeutic intentions were overpowered by events. Here I part completely with the social constructionists who claim that psychiatry’s apparent good intentions were a sham, a pretense for gaining professional power. The history of the

asylum era is the story of how progressive and humane aspirations became relentlessly and repeatedly disappointed. The many well-meant initiatives of asylum psychiatry, central in the legacy that psychiatry's past has lent to the present, were almost all doomed to failure under the pressure of numbers.

In 1800, only a handful of individuals were confined in asylums. Beds in even the most famous of historic asylums, such as Bedlam in London, Bicêtre in Paris, or the "fools' tower" (Narrenturm) in Vienna, numbered in the dozens or low hundreds. In the nineteenth century, these numbers exploded. By 1904, there were 150,000 patients in U.S. mental hospitals, corresponding to a rate of almost two for every thousand population.[1] By 1891, France had 108 asylums.[2] A few years later, the London region alone counted no fewer than 16 asylums, including such giants as the London County Asylum at Colney Hatch ("Nut Hatch") with 2,200 beds, and the Hanwell Asylum (where in the 1840s John Conolly had publicized the nonrestraint of patients) with 2,600 beds.[3] The index of Heinrich Laehr's directory of asylums for German-speaking Europe published in 1891 contained no fewer than 202 entries for public asylums and 200 more for the private sector, to say nothing of the numerous institutions for the alcoholic, the morphinists, the epileptics, and the mentally retarded.[4] In a century's time, the confining of patients with a psychiatric illness had passed from an unusual procedure, born of grave necessity and done only in cities, to society's first response in dealing with psychotic illness.

National Traditions

As the physical platform for the practice of psychiatry, these asylums are central to the history of the discipline. They were founded everywhere at approximately the same time, in countries vastly disparate in social structure and level of economic development. As psychiatry arose in the United States in the early nineteenth century, it was in a sprawling, decentralized land with a tradition of voluntarism in the provision of public services. There would be no national regulation of psychiatric care until after World War II. The geography of early American asylums reflects no logic more than a series of local and adventitious initiatives: Spring Grove State Hospital in Catonsville, Maryland, founded in 1798; Eastern State Hospital in Lexington, Kentucky,

in 1824; Manhattan State Hospital in New York City, in 1825, and so forth.[5] In Britain as well, voluntarism rather than state intervention was the rule. Despite a law of 1808 empowering the counties to raise asylums, the regulation of psychiatric institutions in Britain remained in local hands until the Lunacy Act of 1890.[6]

Traditions of health care on the continent of Europe, by contrast, were very different. In France, state medicine had established itself with the general hospitals of the seventeenth century, followed by the county medical officers of the eighteenth. The rule was that the nation's health was directed from Paris. Extreme centralization characterized every aspect of health care in France, psychiatry included, and the establishment of asylums in the nineteenth century would occur from the top down (not from the bottom up, as in the United States). In Germany, Austria, and Switzerland traditions of state medicine reached far back in time. The notion of central control of medical matters was embedded in such concepts as "medical police" popularized by the Badenese physician Johann Peter Frank in his four-volume work of that title published between 1779 and 1788. Medical police included psychiatry. It was Frank who, after moving to Vienna, ordered in 1795 that a garden be laid down about the *Narrenturm* so that the patients could stretch their legs.[7]

All this administering was done in Central Europe, as in France, from high to low, from central chancelleries down to the asylum doctors in small towns on the periphery. The difference between France and Germany, however, is that France was a single, central state and Germany before 1871 a confederation of 39 separate states, each having its own tradition of administrative centralization. Some of these states led in founding asylums, others lagged. Prussia, by far the largest (except for Austria), was an innovator. Yet Bavaria, with one of the continent's renowned medical schools in Munich, lagged. A host of smaller states such as Baden, Württemberg, and the several "Saxonies" had universities with brilliant and far-flung international reputations, and played roles in the history of psychiatry quite disproportionate to their size.

Germany became the world leader in psychiatry during the nineteenth century precisely because of this dispersal of academic talent into many separate universities, each nurtured by the dynastic ambition of its own little principality. Germany possessed some 20 separate universities in addition to two medical academies, each struggling for glory and competing in a lively

race for scientific advancement against the others. In almost every one of these little lands, academic talent was hooked up to asylum administration. The ministries of interior and education simply decreed that it would happen. France by contrast had basically one university, the Sorbonne in Paris. And even though some of the provincial cities had faculties of their own, they were dim bulbs compared to the great concentration of academic talent in Paris, the only place that mattered. Even today, Parisians indifferently lump the rest of the country together as “la province”; then as now, if one wished to have an important scientific career, it could be only in the City of Light.

The proliferation of German states also made the asylum psychiatrists in the various states more competitive against one another. All were avid for state honors. The German states had a long tradition of recognizing deserving officials and professors with almost meaningless titles, such as *Geheimrat* in Prussia (privy counsellor) or *Hofrat* in Austria (court counsellor). Seeking to demonstrate their devotion to duty, these asylum directors were constantly trying to innovate, reform, and thus distinguish themselves. The narrative of reform would thus become heavily German, in contrast to France where it was pointless to seek distinction in some provincial nest.

Despite these national differences, psychiatrists began everywhere early in the century with the same high intentions. Virtually to an individual, they intended to implement the principles that the founding generation had enunciated of daily regimen and moral therapy to foster the cure of psychiatric illness. One may distinguish between isolated early initiatives that led nowhere, such as Chiarugi’s in late-eighteenth-century Florence, and the launching of a sustained wave of therapeutic asylums.

Sustained was the German wave. The new-style asylums gathered momentum earliest in Central Europe where, in the first decade of the nineteenth century, ministerial officials decided to give Pinel’s and Reil’s ideas a try.[8] This occurred first in the Kingdom of Saxony as the government decided to separate the hospice patients from the criminals. The Saxon government sent a family doctor named Christian August Hayner to study with Pinel, then in 1806 appointed him chief physician of a hospice at Waldheim. Entirely typical of its day, the Waldheim facility mixed together “curable and incurable mental patients, epileptics, the physically handicapped of every description, orphaned children, and criminals of the most diverse kinds in a chaotic mish-mash.”[9] Hayner wanted to separate them, in particular dividing the curable from the incurable insane. In 1808, the Saxon

government asked him to investigate the possibility of doing so by converting the fortress at Sonnenstein into an asylum for the acutely ill; two years later Hayner produced a proposal.

This kind of separation seemed like a clever idea. But in fact, the notion of making progress by separating the fresh cases from the chronic was somewhat of a will-o'-the-wisp.[10] Patients tend naturally to remit from such illnesses as depression and mania and even in many cases from schizophrenia, in contrast to dementia from which they do not recover. So most of the psychotic patients were potentially "curable" in that they would sooner or later get better anyway.

In 1811, the government opened Sonnenstein. Yet to Hayner's dismay, it was not he who was chosen as director but another psychiatrist, the 34-year-old Ernst Pienitz. Pienitz too had done a French apprenticeship, traveling to Paris at the same time as Hayner to see Pinel and Esquirol. (When Pienitz married a Parisian, Hayner and Esquirol were witnesses.) Pienitz had also studied in Vienna under Johann Peter Frank, and at the *Narrenturm* he had accompanied the director on rounds. Pienitz thus reflected the same liberal, humanitarian spirit that ran through this whole generation of physicians.

Cure of mental illness was the number one task of the Sonnenstein asylum. It corresponded to all of the "therapeutic asylum" notions of the time, containing a billiard room, gardens, music rooms with three pianos for concerts once every two weeks, and reading rooms that contained serious books ("no silly French novels").[11] Pienitz relied extensively on baths, or hydrotherapy. He was at great pains to find trustworthy attendants who would not beat the patients. He would make rounds with a surgeon (and a cleric), trying to remedy, in an across-the-boards assault on human unreason, any complaint at all that he could get a purchase on.[12] Pienitz did his task well. The Sonnenstein became celebrated as the "rising sun" of the new psychiatry, and he was rewarded for his efforts by becoming later in life an honorary "Privy Medical Counsellor" *(Geheimer Medizinalrat*).[13]

There is one other interesting theme in the Sonnenstein story. As part of the new spirit starting to blow through the private sector, around 1811 while Pienitz was director at Sonnenstein, he started taking patients into his home. He then established in the nearby city of Pirna an exclusive private asylum of about 20 beds, the third such in Germany. In the years ahead, reforming psychiatrists all over the Atlantic community would use such private establishments as a kind of clinical home base.

Meanwhile in the Rhineland, part of Prussia, another of the pathbreaking asylums was opening its doors. Prussia possessed, just as the Kingdom of Saxony, the same kind of interventionist senior administrators who saw in psychiatry a means of health reform and were willing to back practitioners such as Hayner and Pienitz. As early as 1805, the bureaucrats supported the efforts of Johann Langermann to reform the asylum in Bayreuth. At the conclusion of the Napoleonic wars, the bureaucrats then returned to this task in earnest. In 1817, Karl von Altenstein, the powerful Prussian minister of education, decided to enlist Langermann to help apply the Pinel-Reil reforms in Prussia as a whole. To start with, they decided to turn the old buildings of the former monastery at Siegburg, about two hours' coach ride from Bonn, into a model hospital for curable psychiatric patients. The ministry chose as chief physician a specialist in state medicine named Maximilian Jacobi.[14]

Jacobi, 45 in 1820, had been educated in the most progressive settings, including Edinburgh, and had already made a name for himself through his efforts to reorganize the Bavarian state medical service. Soured at state medicine, Jacobi decided to try his hand at psychiatry and came into contact with Altenstein. In 1820, Jacobi did a tour of eight German asylums, after which the ministry let him take over the planning of the model asylum at Siegburg, which opened in 1825.[15]

It was really Siegburg and not Sonnenstein that became the beacon of psychiatric reform in Central Europe. Jacobi, unlike Pienitz or Langermann, published widely. And he managed to bring to bear therapeutically more than baths and good intentions. Jacobi applied almost to the letter the prescriptions of the founding generation about the restorative nature of an orderly regimen and the psychological impact that the physician himself, as a model of decency and duty, should have on the patients. His 1834 book on the organization of the asylum was translated into a number of languages, and motivated physicians from far and wide to come to Siegburg to observe. Jacobi conceived the asylum as a "hospital organized for the exclusive treatment of those organic illnesses that are associated with mental illness." Like any hospital, it had to possess a medley of therapeutic interventions, going beyond medication to include "baths of all kinds, electricity, galvanism and so forth, the whole array of the relevant treatments that belong to psychiatric as well as to somatic therapeutics, to which must be added diet, healthy air, suitable temperature, and keeping patients physically active and busy."[16]

How did Jacobi actually proceed with patients? Here is Heinrich N., “a strapping big, powerful farmer,” who around the age of 39 began having periodic attacks of psychosis that would remit after a few weeks, leaving him in full possession of his senses until the next attack. Jacobi treated him with the standard medical therapies of the day, bloodletting, laxatives, and the like. But Jacobi also spent a good deal of time on nonmedical measures, such as worrying about the patient’s diet, talking with him to win his confidence, physically laying-on hands by taking his pulse, and negotiating with him. When Heinrich raged in an attack, smearing his feces, Jacobi threatened the straitjacket and isolation room. This “greatly embittered” the patient, “who finally declared that he would behave peacefully and keep clean if we would agree not to use restraints. We gave in to him, and he kept his word. A few days later, he was decidedly on the path to convalescence.” Later Jacobi was chatting with Heinrich about what seemed a psychotic fear of being burned alive. “He gave the following explanation. Earlier when he was in an agitated state, people would chain him fast in the barn, with straw for his bed, and hang a lamp directly overhead. He had always been gripped by the fear that a spark might fall down and burn him up with his bed, a fear of burning alive that accompanied him continuously thereafter.”[17] Although there are features in Jacobi’s management reminiscent of the traditional asylum, with its restraints and efforts to adjust the humors and irritations supposedly causing the illness, this use of the doctor-patient relationship therapeutically was thoroughly reformist, employing persuasion and the orderliness of a daily schedule to effect what he hoped would be a cure.

Siegburg’s golden era occurred under Jacobi, who died in office in 1858. (Three years earlier, he had become a *Geheimer Medizinalrat*.) Psychiatrist Karl Pelman, who became a superintendent elsewhere and once had been an assistant physician at Siegburg, reflected about those halcyon days in the history of German psychiatry: “One cannot begrudge it to an asylum director if his thoughts travel back involuntarily to the old and in this respect better days, as he sits for hours at his desk and answers questionnaires whose purpose and sense are never quite clear to him.... In all my time at Siegburg, I never had to give a forensic expertise and nobody had yet invented [psychiatric reports on] accidents. We had all kinds of time, and we had to dedicate a part of this time to the patients, something that today is impossible even with the best of will.” Pelman remembered Siegburg as having almost the intimacy of family life: “The whole asylum really was a kind of family,

and everybody shared in the ups and downs of the individuals. Even patients in the agitated ward would feel a kind of obligation to be calm and make less noise if there had been a tragic event in the lives of one of the doctors' families."[18] It was in this kind of spirit that the reformed asylum in Germany was launched.

In France, the wave of reformist asylums washed scarcely beyond Esquirol's Paris. Asylums in the French provinces remained desolate custodial institutions, barely a step up from jails with no progressive energy or thoughtful experimentation.[19] The difference between Germany and France is striking. In decentralized Germany, where everything was "province," local centers of innovation such as Siegburg and Sonnenstein had sprung up, jewels in the crowns of petty rulers. By contrast in centralized France, where a single haughty city confronted a vast provincial desert, neither Esquirol himself nor the administrators of the Ministry of the Interior in Paris cared much for what was happening outside. As historian Jan Goldstein puts it, "Esquirol took as axiomatic that all expertise in France about the treatment of lunatics was to be found in Paris and that any improvement in treatment procedures must necessarily take the form of an exportation of that expertise from the Parisian center to the benighted periphery."[20]

Thus Esquirol was able to shape events within the capital until his death in 1840. Becoming chief physician in 1825 of the Charenton asylum, which accepted patients referred by the state as well as family-supported patients, Esquirol was soon able to give the institution a handsome international reputation. He erected clean new quarters for the female patients, to which he attributed a salutary influence in improving the cure rate. For the private patients, he arranged "a salon where they may give themselves to various sociable games, to music and to dance, among each other and members of staff." Patients also had billiards and a large garden in which to stroll about. The men might go out on day passes. ("The women never go out alone.") The nonpaying patients as well had special gardens and activities, the women sewing, the men practicing military drills. "Thus organized, the Charenton institution offers very favorable conditions for the treatment of the insane," said Esquirol. What precisely was so therapeutic at Charenton? "The pleasant locality, the dependability and progressive spirit [*douceur*] of the administration, the zeal of the physicians, the ample provisions of care, the general atmosphere of the place ...": All had made mental illness at

Charenton treatable, he claimed.[21]

Yet in France, mental illness became “treatable” in this sense only at Charenton. No further wave of douceur radiated out. Bureaucratic inertia and political resistance stymied all of Esquirol’s efforts to export his reforms to the 86 departments. Although Esquirol did place several of his students in outlying asylums—Achille-Louis Foville, for example, at the Saint-Yon asylum in Rouen (from which institution the fateful doctrine of degeneration would later emanate)—provincial France remained far behind provincial Germany.

Finally in 1838, the French bureaucracy contrived a national law regulating the administration of asylums in Paris and the departments. The law focused mainly on mechanisms of admission to an asylum (making a preliminary court order unnecessary), and aimed at extending a network of asylum services across the country. Moral therapy was definitely not the focus of these reforms, attracting little attention even in parliamentary debates. [22] Half a century later, many areas of France were still without public asylums, patients ending up in run-down private asylums resembling warehouses.[23] On the whole, the entire mental health apparatus created by the law of 1838 offered but a minimum of care. As the rising tide of patients overwhelmed asylum services in every part of Western society, in France it must be said there was little to overwhelm.

Britain resembled Germany for its lack of centralization. The British state bore as a birthmark its almost complete uninvolvement in healthcare and in medical research, offering the English a government so lean as to limit its powers to collecting customs and policing the streets. For this lack of engagement in medical matters, the English would later pay a price in laboratory research. (Even though state power in Germany was fragmented, each of the small principalities possessed age-old directorial reflexes, the complete opposite of Britain.) Yet a crucial issue in the diffusion of new ideas in medicine and psychiatry is whether the center overwhelms the periphery. In France it did. In England and Germany it did not. The scurrying administrative and industrial towns of England and Germany offered a more permissive terrain than did the ossified ministerial bureaucracy of the French state, which was bound to find moral therapy, with its implications of liberty for the patients, a nuisance.

Among English alienists in the first three decades of the nineteenth century, the concepts of moral therapy and the therapeutic use of daily

routine positively blossomed. As John Ferriar, a general physician who also oversaw the Lunatic Hospital in Manchester, wrote in the 1810 edition of his *Medical Histories*, "A system of mildness and conciliation is now generally adopted, which, if it does not always facilitate the cure, at least tends to soften the destiny of the sufferer."[24] The moral treatment of William Tuke had become an English watchword. In 1813, William Tuke's grandson Samuel published a widely read memoir on exactly how the therapy worked: "The desire of esteem is considered at the Retreat as operating in general still more powerfully [than fear of punishment]. This principle ... is found to have great influence, even over the conduct of the insane." Learning to overcome one's "morbid properties" resulted in "strengthening [the patient's] mind and conducing to a salutary habit of self-restraint; an object which experience points out as of the greatest importance in the cure of insanity by moral means."[25] These were classic lines in the history of psychiatry, even though they were written by a layperson. News of the York Retreat traveled all over the United Kingdom and the continent, a reminder of what could be done.

Other reformist impulses came to Britain from the Continent. Asylum-owner George Man Burrows, for example, seems to have drawn his psychiatric liberalism from Europe. In 1816, he retired from general practice, where he had become a prominent medical spokesperson, to open a small private asylum in Chelsea. The following year, he visited the Parisian psychiatrists, to return to England where in 1823 he founded a more substantial private psychiatric hospital in Clapham that Burrows called "the Retreat." Burrows was among the first to prefer the term "asylum" to "madhouse," and gave, in a textbook on mental illness published in 1828, an account of the standard techniques of gentle management as then widely practiced on the Continent. "If the moderns have any claims to preeminence in the cure of insanity," he declared, "it is certainly from studying those means which have been denominated moral." Among the guidelines Burrows enumerated, such as not trying to argue acutely ill patients out of their symptoms, he included therapeutic use of "the soothing voice of friendship to calm the agony which reminiscence often generates."[26] The soothing voice of friendship: how appropriate a therapeutic mode for upper-middle-class doctors to deal with the kind of upper-middle-class patients enrolled in Burrows's asylum. And yet such techniques were effective. The textbook was "by far the most complete and practical treatise on insanity that had then appeared in this country."[27]

The English psychiatrist who described the new approach in greatest detail has nearly passed from memory because subsequent generations of medical memorialists found him as a phrenologist—a believer in the doctrine of diagnosing mental illness from the bumps on the skull—some—thing of an embarrassment. William Charles Ellis was the founding superintendent of two new county asylums—the West Riding of Yorkshire Lunatic Asylum at Wakefield (beginning in 1818) and the Middlesex County Lunatic Asylum in London (from 1831 to 1838). He emerged as the principal Continental-style innovator in England. Conceiving his institution as a large family in the manner of Esquirol and Jacobi, Ellis saw asylum therapy as applied humanitarianism. As he wrote in 1838, "The moral treatment is by far the most difficult part of the subject. In this the most essential ingredient is constant, never-tiring, watchful kindness: there are but few, even amongst the insane, who, if a particle of mind be left, are not to be won by affectionate attention." And it worked, he said. "In many cases, there will be the delight of witnessing the gradual return to reason and happiness."[28]

In imitation of Edward Parker Charlesworth who had initiated "the nonrestraint system" at the Lincoln Asylum in 1821, Ellis at Hanwell abolished the policy of restraining agitated patients. In the manner of Horn at the Charité, he and his wife introduced crafts and activities with which to fill the day. By 1837, three-quarters of the 612 patients at Hanwell were doing some kind of useful daily work.[29] Even Ellis's eccentric doctrine of phrenology became an extension of moral therapy, for in running his fingers over his patients' heads and talking to them, Ellis conducted a highly therapeutic laying on of hands, calming and comforting them in a manner that previous medical therapy, with its purgatives and bloodletting, had not accomplished.[30]

If the Hanwell asylum in London was the English epicenter of moral therapy and busy regimen, Dumfries was the Scottish. Casting about for ways to deploy the inheritance of her late husband in the relief of madness, Elizabeth Crichton of Dumfries happened upon William Alexander Francis Browne's book *What Asylums Were, Are and Ought to Be,* published in 1837.[31] Browne was then at the nearby asylum at Montrose in Scotland. He had studied under Esquirol at Charenton, and as head of Montrose in the 1830s became an early partisan of the abolition of physical restraints.[32] Browne was filled with the spirit of the curative asylum. "The whole secret of the new system," he wrote in 1837, "and of that moral treatment by which the

number of cures has been doubled may be summed up in two words, kindness and occupation."[33] This was precisely what Mrs. Crichton was looking for. She sought Browne out at Montrose, then donated £100,000 for a 120-bed hospital, the Crichton Royal Institution, that incorporated these principles and accepted patients from all classes.[34] It opened in Dumfries in 1839 with Browne its first medical director.

Thus by 1839, the new wave of asylums had well established itself in Britain. Here as on the Continent the notion became implicit that the function of the asylum was to cure, that of the alienist to use all the techniques implicit in the doctor-patient relationship and in the management of time to alleviate illnesses caused at bottom by disorder of the brain.

America lay distant on the periphery and came last as the reform wave broke across the Atlantic community. As in Europe, the alienists who carried forward this first wave tended to be experienced physicians, animated by a vision as intense as that of Jacobi or Ellis. By 1811, it had become apparent to the members of the Society of Friends in Philadelphia that the insane ward of Pennsylvania Hospital, to whose foundation they had in 1752 contributed so materially, was growing too crowded. They therefore resolved to build a regular asylum on a patch of land they bought in nearby Frankford, Pennsylvania. In May 1817, this first incursion of the new wave in America opened for business. Its model was the Retreat in York, England. As a later annual report of the Frankford Retreat boasted, "This asylum was the first erection of that kind on this side of the Atlantic in which a chain was never used for the confinement of the patients." At Frankford, as under similar regimens all over Britain and the Continent., the rule was, "Come what may, the law of kindness must prevail." [35] Unlike the Retreat at York, that at Frankford had a resident physician, Charles Lukens, from the beginning.

Now a series of foundations ensued. Impressed by the asylum at Frankford, Doctors Eli Todd and Samuel Woodward decided to animate the state medical society of Connecticut to erect a similar institution, or Retreat, at Hartford. A number of individuals subscribed to this quintessentially Anglo-Saxon kind of voluntary institution, and in 1824 the Hartford Retreat, semipublic in nature but open to the poor from Hartford, began admitting patients. Todd, a Yale arts graduate who like most of the physicians of his day had learned his medicine as an apprentice, was most active in the subscription and was appointed superintendent. Woodward, also a Yale graduate who had apprenticed to his physician father, consulted at the Retreat

and to the penitentiary in nearby Wethersfield. Thus it was Todd's philosophy that guided the Hartford Retreat. Todd made, he said, "the law of kindness the all-pervading power of the moral discipline of the Retreat and required unvaried gentleness and respect to be manifested towards the inmates of the institution by every member belonging to it."[36] The Hartford Retreat later became entirely private, underwent several name changes, and exists today in that city as an exclusive private nervous clinic called the Institute for Living.

Almost simultaneously, Massachusetts acquired an asylum. Around 1810, the merchant princes of Boston decided to build a voluntary general hospital. But as events became protracted, the asylum that was to have been part of that hospital opened independently in 1818, even before the hospital itself (the Massachusetts General Hospital began accepting patients only in 1821). In 1826, this psychiatric service was renamed the McLean Asylum after the gift of John McLean, a Boston merchant who left the institution a sum of money. The first superintendent of McLean was Rufus Wyman, a Harvard graduate who had apprenticed in medicine in Boston. Wyman had read Pinel and Tuke and became a strong advocate of "the system of moral management." In his first report in 1822, he described the benefits of occupation and diversion in the regimen of the asylum: "Amusements provided, as draughts, chess, backgammon, nine-pins, swinging, sawing wood, gardening ... divert the attention from the unpleasant subject of thought and afford exercise both of body and mind." To procure for the patients some outdoor exercise, Wyman arranged in 1828 for the asylum's first carriage and pair of horses. He later boasted that "chains or strait jackets have never been used or provided in this asylum" and that no attendant was permitted to lay hand upon a patient. (In fact early patients at the McLean Asylum were restrained.)[37] Like the Hartford Retreat, the McLean Asylum later went private once public beds for psychiatric patients became available in Massachusetts.

At this point, the baton fashioned by the private sector passed to the public. The first public asylum of any kind had been founded in Williamsburg in 1773. The first to arise from the new wave of therapeutic management arrived some 60 years later at Worcester, Massachusetts, proposed to the legislature in 1830 by the educational reformer Horace Mann. In January 1833, Worcester opened for business, its superintendent Samuel Woodward having come up from Connecticut where he had been a consultant for the

Hartford Retreat. (Only in the United States did reserves of experience accumulated in the private sector fuel the public.) Woodward was not exactly a beacon of therapeutic *douceur*, it must be said. He blistered his patients' skin with caustic compounds in the hopes of getting the poisonous humors to the surface. He locked them in the strong room when they misbehaved. Nonetheless at Worcester, he brought tranquillity and orderliness to the lives of the "raving lunatics." "Of forty persons who formerly divested themselves of clothing, even in the most inclement seasons of the year," wrote the hospital's trustees in 1833, "only eight do it now. Through all the galleries, there is far less susceptibility to excitement, more quietude, more civility and kindness exercised towards each other. The wailings of the despondent and the ravings of the frantic are disspelled."[38]

Now many American public asylums began to dispel the wailings of the despondent. A number of asylums having therapeutic rather than custodial intentions were established in the 1840s, ranging from Utica State Hospital (where Amariah Brigham, cofounder in 1848 of the organization that ultimately became the American Psychiatric Association, held forth), to an asylum in Milledgeville, Georgia, that initially possessed the noblest of therapeutic intentions. Its superintendent would personally release new patients from manacles as they were brought for admission. The Milledgeville asylum later became an 8,000-bed hellhole.[39]

Thus by the 1840s, therapeutic asylums had burst forth all over the Atlantic community. In Europe as well as the United States, these young psychiatrists smelled the scent of victory in the air. In their reformed institutions, they were about to conquer mental illness.

The Pressure of Numbers

The reformers were defeated not by the faulty nature of their concept but by the pressure of numbers. The therapeutic asylum bore within it the seeds of success, for people with major psychiatric illnesses are indeed helped by sheltering in places they believe to be safe, by efforts to help them organize their time and lives, and by medication. The early asylum attempted all of these, yet under the assumption that physicians and attendants would be able to spend time treating patients rather than simply warehousing them. What

happened was the overwhelming of the therapeutic asylum by numbers. By 1900, any hope of achieving the early reformers' ideals had been dashed by the flood of inmates hurled against the gates.

In the United States, the first pessimistic note was struck in 1869 with the establishment of an asylum in New York State only for chronic patients: the Willard State Hospital, the first American institution to abandon any pretense of cure and discharge.[40] Under the pressure of numbers, there were many Willards: average annual admissions to the typical American asylum climbed from 31 in 1820 to 182 in 1870, the average number of patients per asylum rising from 57 to 473.[41] Already in the 1870s, observers were transfixed at the remorseless rise in numbers, the unceasing need for ever more asylums. Said one New York banker to a visiting English psychiatrist in 1875 about the continuing demands, "I don't know how it is. They have cost enough, but we never know how the money goes. There is always a tap leaking somewhere."[42] After the 1880s, most American public asylums would abandon any effort at therapy in what historian David Rothman has described as "the decline from rehabilitation to custodianship."[43]

In 1895, the young Swiss psychiatrist Adolf Meyer found himself at the famous Worcester Asylum—birthplace of public-sector therapy—as a research scientist supposedly without clinical responsibilities. There were four other physicians at Worcester attending 1,200 patients, with 600 new admissions a year. Meyer protested this extreme work load (300 patients per doctor) and soon obtained a doubling of the medical staff. "How do you keep the boys busy?" a medical visitor asked him.[44] In other words, the expectation of being therapeutic had clearly vanished from the job description of the typical American psychiatrist.

Psychiatrists in Germany were no less hammered by the pressure of numbers as the rate of confinement grew from one psychiatric inpatient for every 5,300 population in 1852 to one for every 500 in 1911.[45] German psychiatrists were no less bewildered than American at this remorseless rise: As soon as one asylum was built, another was needed. "Thus the lack of beds has become the permanent preoccupation of almost all mental-health authorities," said one doctor in 1911.[46] Another lamented, "It is unsettling how patients needing institutional care are increasing, and bears no relationship to the increase in population."[47] Of Upper Bavaria, one wag said in 1907 that, if things persisted at the present rate, in 222 years the entire

population of the province would be in an insane asylum.[48]

By 1911, the 14 pavilions of the Sainte-Anne asylum in Paris, originally designed in 1867 for 490 patients, housed 1,100. When Vaucluse asylum in nearby Epinay-sur-Orge opened in 1869, it was slated for 500 patients; by 1911 it had over 1,000. The same unanticipated overcrowding held true for almost every asylum in the Paris region founded after 1867.[49] Meanwhile in the classic Parisian asylums, overcrowding had worsened conditions desperately. A visitor to Bicêtre in the 1880s described "this melancholy conglomerate of overcrowded rooms and damp courtyards."[50]

In England, asylum inmates had more than doubled from 1.6 per 1,000 population in 1859 to 3.7 per 1,000 in 1909. In 1827, the average asylum had housed 116 patients; in 1910, the number reached 1,072.[51] Just before World War I, the head of the visitors' committee of one of the county asylums in Staffordshire noted in the register, "Our numbers have increased out of all proportion to our accommodation. On Saturday last we had the largest number, 916 patients, that has ever been in this asylum.... We have 36 males sleeping without bedsteads. I have ordered 20 iron bedsteads at a cost of 30/3d. each."[52] Montagu Lomax, a family doctor seconded to help out in an asylum during World War I, later reflected how completely English asylums had abandoned any kind of therapeutic role. "Our asylums detain," he said, "but they certainly do not cure. Or if they cure, it is only by accident, so to speak, and in spite of the system, not as a result of it." He never had fewer than 350 to 400 patients under his care, a number that "was sometimes doubled or trebled, and it was simply impossible to give them all individual attention." Had the superintendent at least maintained the hope of therapy? "If remedial treatment and methods of attempted cure were considered part of a Medical Officer's duties, I can only say that, as far as I was concerned, I received no hint of the fact during the whole of my term of office." Lomax concluded that, "public asylums for the most part exist merely to confine, not to cure, the insane."[53] The former hopes of a Battie or an Ellis had been shattered by the press of bodies.

Why the Increase?

Why this increase in numbers of asylum patients occurred is highly controversial within the social history of psychiatry. Several distinct schools have emerged. Dominating the field for the past two decades have been scholars who doubt the very existence of psychiatric illness, believing it to be socially constructed. These writers have attempted to trivialize the illnesses of the inmates and to make the case that capitalist society was venging itself on the patients for their unwillingness to work, for a Bohemian lifestyle, or even for a revolt against male authority. Thus society's growing intolerance of deviance is said to have led to the confinement of ever greater numbers of "intolerable" individuals.[54] It is astonishing that this interpretation could have achieved such currency as there is virtually no evidence on its behalf.

Another group of researchers argues that, while psychiatric illness is indeed very real and not necessarily an artifact of labeling, its incidence probably does not change very much over time. Therefore social explanations must be sought for the great nineteenth-century lockup.[55] This model is a serious contender, yet it is weakened by these researchers' reluctance to decompose "madness" into its component parts and see what happens to each. Yet differentiating is essential. To write a history of psychiatry that did not distinguish among dementia, psychosis, and feeblemindedness would be like trying to write a history of noise that did not distinguish between that made by computers and by tanks. Some psychiatric illnesses may well remain historically constant, others do not. Speaking in an undifferentiated way about lunatics, the insane, the distracted, and so forth, renounces at the outset any effort to peer beneath the surface of labels. The search for social causes therefore begs the question, causes of what, exactly?

A third group of writers argues that psychiatric illness is real and that it can change in frequency depending on social circumstances that might affect mind and brain.[56] I belong to this school. In my view, it is essential to disaggregate "madness," and to examine the many different diseases and syndromes whose final common pathway is dysphoria, psychosis, or dementia. The massive increase in the number of asylum patients during the nineteenth century seems to have had two components: a "redistribution effect" and a genuine increase in the rate of psychiatric illness. Understanding these requires penetrating the monolith of madness to see the people and their conditions underneath.

Some of the increase in asylum admissions was a result of redistributing the ill. During the nineteenth century, individuals with major psychiatric

illnesses were increasingly shifted from the family or the poorhouse to the asylum. This redistribution had nothing to do with the overall rate of mental illness. It simply involved rearrangements in care.

In addition, however, some kinds of psychiatric illnesses increased in frequency. During the nineteenth century, several major components of "madness" were on the rise, in particular neurosyphilis, alcoholic psychosis, and apparently, though this is less certain, schizophrenia.

The historical evidence of the authenticity of both of these processes—the redistribution of psychiatric patients and their rising numbers—is so overwhelming that it may no longer be ignored on the grounds that it is ideologically inconvenient. Many of the psychiatric historians who came of age in the 1960s and 1970s have constituted a kind of lost generation in that they have chosen to pursue puffs of smoke, displaying no interest in the question of just what happens historically to make mind and brain go awry. If we wish to tell the history of psychiatry empathically, we must deal with the story of illness rather than arguing that it is a nonstory or that it is unknowable.

Redistribution of Illness

Consider the plight of the impecunious family with a mentally ill relative at home. We are in nineteenth-century Vienna. "If the poor have to retain their mentally ill relatives in their overcrowded homes for a considerable period of time," said Viennese psychiatry professor Julius Wagner-Jauregg in 1901, "many family members suffer from interruptions of their night's sleep, or become fearful or exasperated by the patient's behavior, or cannot afford appropriate medical attention because money is short. A crisis is created. These are the people who bear the brunt of the overfilling of the asylum."[57]

Looking after the mentally ill fell in the first instance to the family. It was the family that decided to keep an afflicted individual at home or to seek care. "For the entire period from 1843 to 1900," concludes one student of American asylum psychiatry, "families continued to dominate the commitment process."[58] Thus, for the number of asylum patients to rise, families would first have to decide to send them away.

But why was it during the nineteenth century that families reached this

decision so often? Was it because there previously had been no asylums to send afflicted relatives to? Or was it because of some change in the dynamic of family life? Was it the push of the family or the pull of the asylum?

Consider the pull argument. It is perfectly true that in the absence of asylums, families would have to keep distracted relatives at home, or send them out on the high road. But that is true only of poor families. Rich families too have always been plagued with the problem of mental illness, and these families could always have afforded to purchase outside accommodation, should they have so desired. Some did dump afflicted relatives on the Church, but we know that mad relatives in upper-class families were most commonly retained at home. The mad princes of Renaissance Germany would be locked in their rooms, or in fortresses.[59] Andrew Boorde, a Montpellier MD and cleric, devoted the "madnesse" section of his *Breviary of Healthe* published in 1552 to the domestic management of the condition.[60] Before a certain time, these wealthy families showed no interest in private madhouses. There were virtually none in England before the eighteenth century, and none before the nineteenth century on the Continent of Europe.

How do we explain the changed willingness of the wealthy to send their relatives away? I think that it is related to changing patterns of sentiment in family life.[61] As the family started to consider itself increasingly an emotional unit, disruptive relatives at home began to seem more and more intolerable. Before the eighteenth century, the family was based more on ties of property and lineage than sentiment. It had little intimacy to disrupt and did not celebrate its unity about the dinner table or at other private moments, of which there were few. Late in the eighteenth century, however, the sentimental climate of family life began to change. Relations became much more intimate, the evening meal an occasion for celebrating the private sentiments of "the little family's togetherness," as the French put it, *la petite famille bien unie.* Insane relatives no longer fit into this picture of bliss.

Bruno Goergen, owner of a newly founded private psychiatric clinic in early-nineteenth-century Vienna, explained why wealthy families had started turning to him. There was something intrinsic in psychiatric illness, said Goergen, that made a patient misinterpret the "consoling words of his relatives, the handwringing and trembling of his fiancée, the tears and sighs of those who are close to him ... as being quite different than what they in reality are. His inflamed imagination, his sensibilities now thoroughly

inimical to any sense of harmony, lead him all too often to see his dear wife for a mixer of poison, his loving children as devils, his agreeable dwelling for a prison. He hears voices that otherwise no one hears, he sees forms that otherwise no one sees.... And in his confusion this man, who otherwise had only a heartful of love for his family, lends neither eye nor ear to their torment, a torment that in gestures and words speaks its name aloud to him."[62] One is hard put to find utterances of this power in either the medical or the lay literature of previous epochs. We are dealing here with a new style of family life, for which mental illness in a dear member is no longer possible to behold.

Statistics show that the greater the disruption, the more rapidly did the family disembarrass itself of ill relatives. Wilhelm Svetlin, owner of a private psychiatric clinic for the wealthy in late-nineteenth-century Vienna, asked families about the duration of the patient's illness before admission to clinic: For 56 patients who came in with melancholia, a third of the families (36 percent) had been willing to wait half a year and more before bringing the patient to the clinic; only 18 percent of the melancholic patients were admitted within a month of less of the onset of illness. For the 16 patients with paranoia, not a single family acted within three months. But of the 22 patients with mania, a disorder that drives its victims restlessly night and day, 68 percent were brought in less than a month after the onset of symptoms.[63] This means that in the 1870s and 1880s, two-thirds of the wealthy Viennese families with manic relatives were unable to endure the patient's whistling, clapping, singing, yelling, and breaking of furniture for more than a month.

The implication of this is that in the 1670s and the 1770s such families were able to tolerate mania. What had therefore changed was not the availability of institutional care, for in the seventeenth century these families could have afforded outside care had they so desired, but the climate of family sentiment itself. The presence of an ostentatiously alienated member was impossible to contemplate within the tight little emotional nucleus that family life had become. Thus one component of the rise in asylum admissions was lessened family willingness to tolerate mental illness. Psychiatric illnesses once treated in the family now became assigned to the asylum.

The same growing readiness to disembarrass probably applied to the demented elderly. Whereas once families were willing to tolerate the presence of a demented senior, by the end of the nineteenth century at the

latest, families started seeking outside care. In 1908, one English medical writer noted an increase in asylum admissions "due to the drafting into asylums of harmless old people, the subjects of senile dementia more or less pronounced, who were formerly kept in workhouses or allowed to remain with relatives or friends willing to look after them ..."[64] At a county asylum in Buckinghamshire, the percentage of people over age 60 at admission climbed from 18.7 percent in 1881 to 24.0 percent in 1911.[65] The same trend held true in the United States. The proportion of patients admitted for "senility" rose sharply at Utica State Hospital in the late 1870s.[66] Data for Warren State Hospital in Warren, Pennsylvania, begin only in 1916, when 14.8 percent of all patients had "mental diseases of the senium." Between 1946 and 1950 the figure was 26.4 percent.[67] Historian Gerald Grob concludes that in the twentieth century, American mental hospitals became increasingly a repository for the elderly.[68] These then are examples of redistribution of illness from the family outward.

Another kind of redistribution involved shipping people previously housed in prisons and workhouses to the asylum. In England, a law of 1874 made imperial funds available for the removal of local pauper lunatics to county asylums, causing a clear shifting of the burden of care.[69] Some writers have argued that these pauper lunatics were more pauper than lunatic: unwanted rather than insane individuals whom the communities were happy to send away.[70] Yet well-informed observers of the day believed that, for the most part, the paupers ending up in asylum had some major psychiatric problem. The annual report of Burntwood Asylum in Staffordshire in 1887, for example, lets us glimpse a superintendent filled with humanitarian enthusiasm, standing at the door of his institution welcoming the latest arrivals from the workhouses: "The admissions from the Workhouses of helpless chronic cases requiring much special care were not a few. Complaint is sometimes made that Asylums should be hampered with these cases; but, for my part, I rejoice to think that the means placed at our disposal for nursing and caring for such poor and afflicted sufferers are happily and beneficially employed in lightening the burden of their troubles."[71]

Not until we have a retrospective analysis of these pauper lunatics and other redistributed patients, case by case on the basis of their files, will we be able to say if the redistributed patients were psychiatrically ill or merely inconvenient. But strong anecdotal evidence suggests that the individuals

thus dumped on asylums by their families and local communities had some serious mental problem, rather than being confined for challenging capitalism, the patriarchal order, and all the other clutter in the storechest of scholarly hobbyhorses that has come down to us from the 1960s.

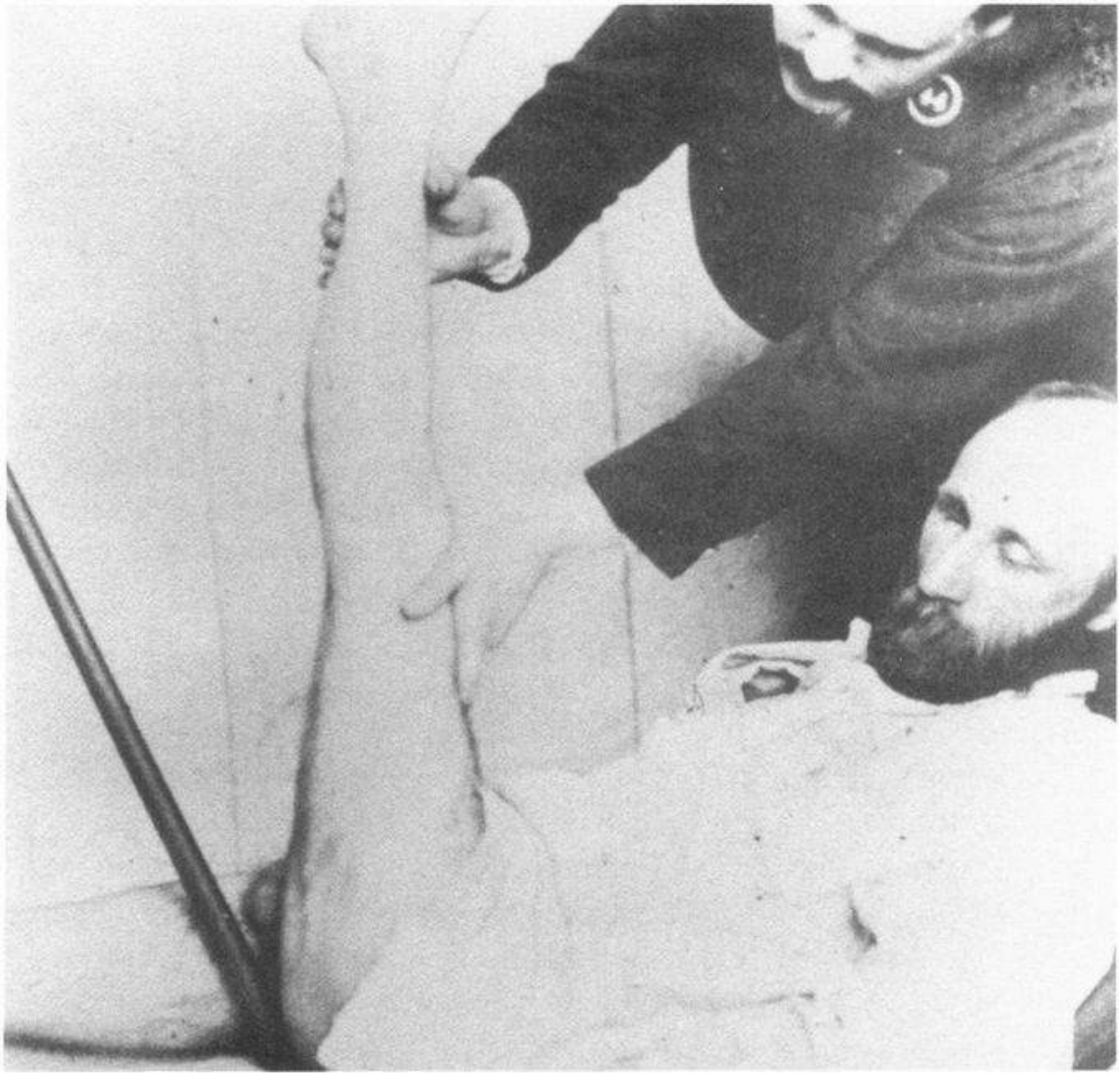
Rising Rate of Psychiatric Illness

A second major component in the press of bodies was a genuine increase in the rate of mental illness during the nineteenth century. Between 1800 and 1900, the risk grew appreciably that the average person in his or her lifetime would be visited by a major psychiatric disorder. Let us take the various elements of this increased risk in order, from the least controversial to the most.

The psychiatric illness that most demonstrably increased in frequency during the nineteenth century was neurosyphilis. The syphilitic infiltration of the central nervous system is of capital importance in the history of psychiatry because it often announced itself clinically in the form of psychiatric symptoms. The end stages of neurosyphilis would be treated in public asylums and private clinics. It is the enormous rise in frequency of this disorder that explains part of the flood of patients into institutions. Once called “the disease of the century,” neurosyphilis has been virtually forgotten today and is routinely ignored by historians of psychiatry, which explains why so much fantastical writing about “the social construction of illness” and the like has passed unchallenged. There was nothing socially constructed about neurosyphilis.[72]

Patient with tabetic form of neurosyphilis in Colney Hatch asylum in London, c. 1900. Note hypotonia of joints, inability to raise eyelids (“ptosis”).

Source: Psychiatry for the Poor by Richard Hunter and Ida Macalpine, 1974. London: Dawson. Courtesy of Dawson U. K. Ltd.



Typically, a young medical student or businessman would have sex with a prostitute, in an era when “nice girls” were not available before marriage. He would experience the sore on his penis or perhaps the swollen lymph nodes of the groin that constitute evidence of primary syphilis. Then the signs of infection would go away, and the episode might pass from his mind. This partly explains why so many later victims in middle age would deny having had syphilis. Not only was it shameful, its first evidence—in an era when skin infections were daily affairs—often passed unnoticed.

But the spirochetes, the microorganisms that cause syphilis, would not leave the young man’s bloodstream. Within a year, they might have invaded

the meningeal lining of the brain and spinal cord though remaining clinically silent. Thus for many years, the young professionals and businessmen who were most at risk of the disease would walk about symptom-free. At this point, one of two things might happen. Either the body's immune system would overcome the disease so that it did in fact go away. Or, a decade now down the road, the infected individuals might start to become symptomatic, in the form, perhaps, of being unable to pronounce some kinds of phrases. In differentiating schizophrenia from neurosyphilis, the young doctor Lewis Thomas in Boston would ask patients to say, "God save the Commonwealth of Massachusetts."[73]

Or the early meningitis caused by the spirochetes might take the form of more frankly psychiatric symptoms, such as the grandiosity characteristic of mania. In one case, a distinguished Frankfurt chemistry professor suddenly interrupted his lecture and started to tell gossipy stories from the city. The previous day he had gone out and bought 10 automobiles and 100 wristwatches.[74] The sudden appearance of psychiatric symptoms of any kind in middle-aged businessmen and professionals made physicians of the day think immediately of neurosyphilis. But the symptom that most set clinicians' teeth on edge was the euphoria of spirochete-induced mania, for the patients always denied illness (they felt great!), and they had the capacity to bankrupt their families (as many did).

As the illness advanced, it might take one of two forms. If it affected primarily the spinal cord, it was known as tabes dorsalis (also locomotor ataxia), or wasting of the posterior part of the spinal cord. Tabes would cause lancinating pains to the abdomen as well as a high-stepping gait that patients described as "walking on cotton." (In the nineteenth century, it was not yet proven that such symptoms were the result of a primary syphilitic infection many years earlier, and so were not called neurosyphilis.) If the disease affected mainly the brain, psychiatric symptoms would be foremost, followed by dementia and paralysis. This form was known as general paralysis of the insane (GPI), dementia paralytica, or progressive paralysis, and was the kind most commonly encountered in the end stages in asylums; middle-class patients with tabes would be more inclined to seek relief at a spa.

Both forms of neurosyphilis were invariably fatal: Once a patient became symptomatic, he or she was finished. As Madame Maria Rivet, the operator of a private nervous clinic in Paris, said in the 1870s, "La paralysie générale ... ne pardonne jamais" (General paralysis of the insane gives no mercy).[75]

Early stages of the disease might appear in a variety of ways, and only physicians skillful at interpreting subtle changes in the pupils, eyelids, or reflexes would be able to spot it. But later stages, especially of GPI, were absolutely characteristic: There is no other disease in which primarily middle-aged men (it affected mainly males) suddenly become demented, then die paralyzed in the presence of terminal convulsions. Thus we may have some confidence that the reported statistics of the time give us a fair indication of the true level of neurosyphilis.

Neurosyphilis is not one of the age-old diseases like melancholia. It seems to have been largely unknown before the last quarter of the eighteenth century. Why this should be so is a mystery, since syphilis has been documented in Europe since the Middle Ages. Yet it was only in the 1780s and after that physicians began reporting the first cases involving the central nervous system.[76] Or what sounds like the first cases at least, for the diagnostic terms "tabes dorsalis" and "progressive paralysis" were coined only in the nineteenth century. Thus William Perfect, author of a psychiatry textbook of 1787, describes a patient whom a colleague told him about: a middle-aged man, "intemperately passionate, and misanthropic to the greatest degree; in this manner his insanity began to appear: he next drew upon his banker for sums immensely beyond what his account would afford, and, when disappointed in this respect, he became sullen, and immediately made out drafts for enormous sums upon houses with which he had not the least connexion." He began to think that he was the Lord Chancellor, King of Spain, and Duke of Bavaria. Thus far the man had shown the standard symptoms of mania. But then he became demented, something that does not happen in mania, and as Perfect's medical friend gave the most recent news, the patient had "gradually dwindled into a total decay as he approached to the verge of idiotism."[77] This sounds like an early case of progressive paralysis.

Almost simultaneously in Florence, Vincenzio Chiarugi was reporting patients who must certainly have had neurosyphilis, such as the 40-year-old soldier, his pupils immobile and unequal (a sign of a brain lesion), who proceeded to become demented, then "completely lost the movement of his lower extremities," dying in bed "of advanced atrophy." Chiarugi also reported a 37-year-old accountant who underwent an attack of mania, then became demented, dying "of a slow marasmus [wasting], having first become almost totally paraplegic."[78]

Within two decades of these early reports, experienced alienists were

recounting as common this combination of paralysis and insanity. Said John Haslam, a physician at Bedlam, in 1809, “A course of debauchery long persisted in would probably terminate in paralysis.... Paralysis frequently induces derangement of mind.” He believed the problem to be on the upswing: “Paralytic affections are a much more frequent cause of insanity than has been commonly supposed, and they are also a very common effect of madness; more maniacs die of hemiplegia [one-sided paralysis] and apoplexy [fits] than from any other disease.”[79] Hence derived the connection between extramarital intercourse and madness that led contemporaries, by association, to warn against the spilling of semen as such, a presumed cause of general paralysis. For their pains, they would be mocked as moralizers by future generations of historians.

By the Napoleonic years, in French asylums this combination of paralysis and dementia had become an everyday occurrence. Esquirol, for example, described the various demented patients he had seen at the Salpêtrière and in his private asylum, the great majority of the private patients and a slight majority of the public ones being under the age of 50. Of these 235 individuals, more than half, he said, “presented some symptoms of paralysis.” He concluded that dementia complicated by paralysis was quite frequent and incurable, but noted no particular difference between men and women.[80]

That was in 1814. Years later, Esquirol had another go at the subject, realizing now that young and middle-aged patients who had become both demented and paralyzed were very common indeed. “Paralysis is commoner among insane males than females,” he said. “Eighteen years ago, when I was in charge of the psychiatric division of Bicêtre ... I was struck by the difference in comparing the insane and paralyzed male patients of Bicêtre hospice with the [far fewer] number of paralytic females at the Salpêtrière.” Esquirol said that his student Achille-Louis Foville, now in charge of the St.-Yon asylum, had noticed this as well: A tenth of all patients at St.-Yon had paralysis: two-thirds of them male, one-third female. And this strange paralysis of the insane was much commoner, noted Esquirol, in the Paris region than in the south of France or in Italy, citing statistics from confreres to prove this point. Yet the paralysis could not have been such a mystery to Esquirol, for in 1826 his young colleague Antoine-Laurent Bayle had demonstrated that both the paralysis and the delusions of grandeur of these patients were a result of chronic inflammation of the meninges, the lining of the brain.[81] These were psychiatric symptoms driven by organic brain

disease, though the cause of the disease was as yet unknown.

Esquirol's and Bayle's observations about progressive paralysis became world renowned. It is often forgotten that in Germany as well, the epidemic of neurosyphilis began its acceleration in the Napoleonic years. In 1814, Christian Friedrich Harless, then professor of medicine in Erlangen, referred to the spinal version of neurosyphilis as "an already long-familiar constitutional illness" known to be fatal once the first signs of paralysis had appeared.[82] Three decades later, the great Berlin neuropathologist Moritz Romberg observed that this disease had recently increased "following the great military campaigns of our era." He baptized it tabes dorsalis.[83]

As an epidemic of venereal disease spread across Europe and North America in the nineteenth century, the epidemic of neurosyphilis lagged by 10 to 15 years, which is the average interval between the original infection and the appearance of psychiatric symptoms. Large numbers of people were involved: Five to 20 percent of the population would have had syphilis on a lifetime basis.[84] Of those, as many as 6 percent would go on to develop neurosyphilis.[85] Six percent of millions is a lot of people. Not all of them would have been admitted to asylums: Some died at home out of sheer shame, others perished in chic watering places like Lamalou in the Pyrenees, or sought suicide with morphine. The asylum population therefore is merely the tip of the iceberg. The point is that the paretics and tabetics bulked large in the life of the nineteenth-century asylum.

Because neurosyphilis tended to be a middle-class affliction, it was well represented in the men's wards of the private nervous clinics: Of 111 male patients in the early 1860s at the private Pöpelwitz clinic near Breslau, 32 percent had general paralysis of the insane; of the 75 female patients, not a one.[86] Among the major psychiatric illnesses at a "sanitarium for nervous and mental diseases" in Kansas City, Missouri, between 1901 and 1907, "general paresis" ranked in frequency only behind depression and mania, and ahead of schizophrenia ("dementia praecox").[87] A dozen other studies from the private sector reflect a similar prominence of neurosyphilis among the major psychiatric disorders.

Neurosyphilis carved its path through the public asylums of the Atlantic community. Before 1850, none of the Jewish patients admitted to the Frankfurt City Asylum had neurosyphilis. By the years between 1871 and 1880, 21 percent did, coming mostly from the class of merchants and other

such paying patients.[88] “When I entered the Toronto Asylum in 1853,” said Joseph Workman, superintendent of the asylum until 1875, “there was not a single case [of GPI], as far as I could judge, in the institution, but it was not long before it began to make an appearance.” In the 10 years from 1865 to 1875, 65 men and 7 women died of paresis. “It is a melancholy confession, but it is the truth, that the asylum records of paresis have been a pretty nearly correct statement of the number of cases of this disease admitted.”[89] England’s Montagu Lomax said that patients with GPI “often form the majority of cases in the male hospital wards.”[90] Thus did the tabetic, whose gait outside in the street heralded his arrival in the doctor’s office, and the paretic, with his jumbled speech and irregular pupils, make their entry in the asylum.

What does all this mean for the social history of psychiatry as opposed to its biological history? Karl Edel’s private psychiatric hospital in Berlin had, in fact, two wings: a luxurious one for private psychiatric patients from the upper orders, and an economy wing for public patients from the city of Berlin and surrounding area. In the late nineteenth century, neurosyphilis at the Edel clinic was very much a middle-class affair: 46 percent of the 976 men who passed through the private men’s ward had neurosyphilis. There were many fewer paretics from the pauper class.

Of women at this clinic, 5 percent of the wealthy and 7 percent of the poor had neurosyphilis.

What is interesting in these statistics from the Edel clinic is what happened to patients after they were admitted with a diagnosis of paresis. Of the private patients, both male and female, around half were picked up by their families to die outside the asylum. The same is slightly less true for the working-class males: Only 30 percent of them were discharged by their families. Of greatest interest is the plight of the 22 working-class women admitted to the Edel clinic with this diagnosis: All died in the asylum. None were picked up.[91] This evidence suggests that the many scholars looking for social significance in the history of the asylum have been barking up the wrong tree: It is not the diagnoses, which tend to represent biological brain events, that are socially constructed but the experiences of the patients with these disorders. It is pointless to claim that psychiatric illness represents merely “labeling,” or social efforts to make deviants conform, when we have these thousands of patients unable to move their legs, with bedsores discharging pints of pus because their attendants do not shift them often enough. Yet in these poor

women, cast by their families into the outer darkness because they have a shameful illness, we have a real story, one alas largely untold.

Another cause of the tremendous rise of asylum admissions was insanity related to alcohol. Drinking large amounts of alcohol can affect the nervous system in various ways: The drug itself can cause hallucinations. Withdrawal from alcohol may produce psychosis, fits, and delirium tremens. And the chronic overconsumption of alcohol may cause a drinker to skimp on other calorie sources, producing a chronic psychosis and memory loss named after Sergei Korsakoff, the Russian asylum doctor who first described it in 1887 (thiamine deficiency is the real cause). Meanwhile Karl Wernicke, a German psychiatry professor and asylum physician, was describing (in 1881) the acute form of the same problem, the mental confusion and staggering gait that may suddenly strike alcoholics. Liver disease from drinking may carry its own psychiatric morbidity. There are, in short, many reasons why a large increase in the amount of alcohol that a population consumes could lead to an upturn in asylum admissions, some short and self-limiting, others long-term.[92]

Drinking increased very substantially in what one historian has called "the golden age of inebriation."[93] In England, the consumption of spirits per person rose 57 percent between 1801 and 1901, from less than half a gallon per person a year to more than three-quarters.[94] While an average adult American drank only 1.8 gallons of absolute alcohol in 1845, the figure had climbed to 2.6 in 1910.[95] Between the end of the eighteenth century and the beginning of the twentieth in France, the total production of alcohol and beer climbed 14-fold: from 1,170,000 hectoliters in 1781 to 16,700,000 in 1913.[96] And in the mid-decades of the nineteenth century, the consumption of beer in Bavaria doubled.[97] Rising standards of living and the cheap production of alcohol from beet sugar were driving this increase, as the French peasant started drinking wine with meals daily and the German craftsman having a bottle of whatever lay ready at hand.

Rising consumption spilled over into the number of patients admitted to asylum with alcohol toxicity. In Prussia, for example, in 1875 there were fewer than 600 such cases, in 1900 some 1,300.[98] Karl Bonhoeffer, later an important figure in German academic psychiatry and in the 1890s a psychiatry resident in the industrial city of Breslau, said of those years, "At that time ... the admission room was dominated by the numbers of alcoholic

patients wandering about in delirium, dragging pieces of bedding with them. On every summer day there was at least one such delirium patient.... It is simply inconceivable to us today what percent of patients in the psychiatric institutions of the large cities had alcoholic delirium....” In the late 1880s at the Charité Hospital in Berlin, 39 percent of all patients had delirium tremens, Bonhoeffer said.[99] Cities like Berlin and Breslau had alcoholism rates far in excess of the population as a whole (given that, between 1875 and 1900, alcoholics were only 3 percent of all asylum patients in Prussia).[100] Nonetheless, compared with earlier times, such psychiatric illness was on the rise.

This increase was not just a German phenomenon. Among the 8,000-odd patients to pass through the forensic psychiatric service of the Parisian Prefecture of Police (the Infirmerie Spéciale) in the years from 1886 to 1888, alcoholism was the number one diagnosis (27 percent of all cases).[101] In the Parisian asylums, a third of the males were admitted because of drink (a tenth of the females). Valentin Magnan, the chief of the Sainte-Anne asylum, believed alcohol to be the main reason for the recent rise in admissions.[102]

In Britain, psychiatry was steeped in ethanol. Between 1874 and 1894, in the various wards of the Royal Edinburgh Asylum, alcoholics formed 15 to 20 percent of all male admissions (the female percentage was much lower).[103] Private clinics dedicated to drying out wealthy alcoholics sprang up all over, such as “Rivermere” for “inebriety and abuse of drugs” near Maldon in Essex, “an ideal private home for the treatment of ladies and gentlemen of the upper class.” Or “Tower House” in Leicester, a “private high-class home for ladies,” again for alcoholics and drugabusers. The *Medical* Directory of the United Kingdom for 1908 carried ads for 24 such “homes.”[104] It is evident that in the second half of the nineteenth century pathological drinking became a problem of considerable psychiatric proportions.

With neurosyphilis and alcoholism, the case is relatively straightforward. Both diseases increased in frequency with obvious consequences for admission to asylum. Yet these two conditions together represented overall only a fraction of all admissions (11 percent for Prussia as a whole in the period 1875-1900).[105] The vast bulk of asylum admissions consisted of patients receiving diagnoses that in retrospect are not transparent, “epileptic insanity,” “hysterical madness,” and the like. To peel back contemporary

diagnostics from the underlying reality of illness, we need something more than these global diagnostic terms of former years: We need to make our own assessments of individual patients. Retrospective diagnosis on the basis of patients' charts is required, making a reassessment from the signs and symptoms reported in the file. Yet this kind of scholarship is very time-consuming, requires a knowledge both of context and of illness, and has only just begun. Therefore what I have to say about the third category of illness, schizophrenia, in which an evident increase in frequency occurred, is somewhat tentative. Yet I think there is enough evidence to justify the conjecture that the incidence of schizophrenia rose significantly during the nineteenth century.

It is now clear that schizophrenia is a genetically influenced disease of brain development, beginning perhaps in the uterus or in the trauma of childbirth as the child's brain does not grow properly, leaving the individual, often from the years of young adulthood on, unable to cope with normal human relationships, to deal with the usual stresses of life, or to organize his or her thoughts successfully (see pp. 268-270). The disease may also result in frank psychosis, meaning hallucinations, delusions, and illusions. Although what is called "schizophrenia" probably represents several different illness processes mixed together—some genetic, some not—there is no doubt that the disease is common, affecting perhaps one percent of the population. In the nineteenth century as well, much schizophrenia doubtlessly figured among the medley of psychotic disorders. But how much? And how does its frequency change over time?

The first descriptions of what is recognizable as schizophrenia appeared in 1809 by Pinel in France and Haslam in England. "Connected with the loss of memory," wrote Haslam, "there is a form of insanity which occurs in young persons": Individuals who previously were of "prompt capacity and lively disposition" start to become silent and incurious. "The sensibility appears to be considerably blunted; they do not bear the same affection towards their parents and relations." They become uninterested in their companions and unable to give any account of what they have just been reading, unable as well to put more than a sentence or two together on paper. Typical adolescent blues? "As their apathy increases they are negligent of their dress, and inattentive to personal cleanliness." They may become incontinent of urine and stool. "Thus in the interval between puberty and manhood," said Haslam, "I have painfully witnessed this hopeless and degrading change, which in a

short time has transformed the most promising and vigorous intellect into a slavering and bloated idiot." Haslam described a young man who had amputated his own penis in a fit of fury. Yet after admission to Bedlam, the fellow appeared to recover. Haslam was mistrustful. "There was something in the reserve of his manner and peculiarity of his look which persuaded me he was not well, although no incoherence could be detected in his conversation." The patient appeared to Haslam to be walking rather lamely and on occasion sat with his shoes off rubbing his feet. He told Haslam that his feet were blistered yet declined to show them to the doctor. One day, again after seeing the patient rubbing his feet, Haslam insisted upon examining them: "They were quite free from any disorder. He now told me, with some embarrassment ... that the boards on which he walked (the second story) were heated by subterraneous fires, under the direction of invisible and malicious agents, whose intentions, he was well convinced, were to consume him by degrees."[106]

The same kind of patients presented themselves across the Channel as well. In the context of "idiotism" among the young, Pinel reported a young sculptor, 28, "previously exhausted by the excesses of intemperance or the pleasures of love," who remained locked in a fixed position, "almost always immobile and taciturn. Sometimes he would give a kind of idiotic and stupid laugh, but there was no expression on the features of his face, and no memory of his previous life. Yet his appetite was never lacking, and even the approach of food would set his jaw to masticating."[107] It is impossible to say if either of these young patients of Haslam and Pinel had the disease that would later be described as schizophrenia. Yet if we find a large number of individuals affected with similar symptoms, it is not unreasonable to conclude that some of them had schizophrenia.

Haslam and Pinel attuned psychiatry to an apparently new kind of illness, one presenting itself as psychosis in young adults and progressing to chronic insanity. Such illness was called "dementia" in the language of the day but it did not in fact entail a loss of intelligence, rather chronic delusions and hallucinations accompanied by disorganized thinking. Descriptions of this kind of presentation are virtually absent from the medical literature before 1800. They become steadily more frequent thereafter.

Historian Edward Hare has called attention to a substantial increase in the number of patients reporting auditory hallucinations—voices speaking to or about them—during the nineteenth century. On the basis of such evidence,

Hare launched the “recency hypothesis”: schizophrenia as a recent disease rather than an age-old accompaniment of the human condition like depression.[108] His answer to the quarrel about whether the rate of insanity increased during the nineteenth century was an emphatic yes: “The condition we now call schizophrenia ... was most likely to have been the main cause of the increased admissions.”[109]

There are really three groups of players here. Hare’s recency hypothesis has been attacked from two directions, by scholars who believe there is no such thing as psychiatric illness, and by a completely opposing camp insisting that schizophrenia has always been with us. It was, according to the antipsychiatry scholars, the madness-making environment of the late-nineteenth century asylum itself that converted individuals whose initial problems may have been mild into “chronic” cases that doctors labeled insanity.[110] According to this interpretation, these patients probably had no disorder at the time of admission and were just victims of labeling and medicalization.

A second group of scholars argues that schizophrenia is “probably very old,” not a new disease at all, and that clear descriptions of it may be glimmed in the patchy premodern medical literature.[111] Thus what changes is merely the exactness and density of the medical literature, and not the phenomenon of disease as it exists in the real world.

A third group advocates the recency hypothesis. Represented by Hare and by some researchers who have undertaken retrospective diagnosis of individual asylum patients’ charts, this school argues that schizophrenia is new and that it increased during the nineteenth century.

This three-cornered debate has stimulated a good deal of research. The stakes are high because the debate is really asking, do the origins of psychiatry lie in the manufacture of illness for reasons of professional gain, or do they lie in caring for a flood of patients afflicted with historically new diseases? Although it is still not possible to resolve the debate definitively, subsequent research has tended to support the recency hypothesis rather than those who argue for the social manufacture of madness.

Relying on nursing notes to diagnose patients retrospectively, one scholar found that descriptions of schizophrenic symptoms were rare in 1790 at the Pennsylvania Hospital in Philadelphia, and that on the basis of similar evidence they had become common by 1823 at Bethlem Royal Hospital. “This lends some more weight,” he concluded, “to the evidence presented

elsewhere for the recency hypothesis."[112]

In the retrospective diagnosis of individual patients' charts, several historians have documented the presence of frank psychotic symptoms that certainly sound like schizophrenia in a number of nineteenth-century private clinics and public asylums. Among children admitted to Bethlem after 1830, the incidence of auditory hallucinations and delusions suggestive of schizophrenia seems to have been on the rise.[113] Of 118 patients admitted to the York Retreat between 1880 and 1884, 31 percent displayed the delusions and hallucinations that would retrospectively qualify them for a diagnosis of schizophrenia. The authors, finding no evidence to support the contention that Victorian physicians "confused insanity with immorality or other forms of nonconformist behavior," concluded that most of the patients admitted to the Retreat in these years were "severely disturbed mentally."[114] Still other studies based on retrospective diagnosis have reached similar conclusions about the prevalence of schizophrenia in the nineteenth-century asylum.[115]

Nineteenth-century physicians who themselves believed insanity in young people to be increasing were probably not so wrong after all. Karl Kahlbaum, psychiatrist and owner of a private nervous clinic in the German town of Görlitz, was among the first to describe chronic madness in the young as a distinctive clinical entity, which he called "hebephrenia" (see p. 104). He observed in 1884, "It must be the experience of all psychiatric institutions that the number of youthful patients has recently undergone a considerable increase."[116]

Maria Rivet, daughter of the distinguished Paris psychiatrist Alexandre Brierre de Boismont, was not a physician, but she had grown up around psychiatric patients. She also ran the family's clinic, a classy establishment in the St.-Mandé quarter. Writing in 1875, she had the definite sentiment that there was something bizarre and unusual about all these young women turning up with major mental illnesses. An example: "Mlle. N. had undertaken a program of study at university that exhausted her mind. Reading medical books also contributed to the commotion in her brain, and despite her youth she is now struck by dementia, whereas this incurable disease generally strikes only the elderly." Wherein lay Mlle. N.'s madness? Initially she believed herself to be Eve, the first woman, "and demonstrated to us in eloquent and evocative terms the splendors of the terrestrial paradise at the time of creation, at which she was present." Then she refused to eat eggs for fear of being changed into a chicken. "The she believed herself to be God, the

sun being the work of her hand, and we had to forcefully prevent her from staring at it. She was outraged, and insulted us for having dared deny her all-power. ‘I created the sun myself,’ she said.” A long train of other delusions followed. Mlle. N. wrote letters from time to time giving voice to these delusional ideas, “but in an almost incomprehensible style.” Recently, Mlle. N. had tamed three of the geese in the clinic’s garden, one of them picking familiarly in her pockets while the other two sat on her shoulders. “She is almost always seated on the lawn, addressing long tirades to these beasts who seem to listen to her.”[117] There are virtually no such accounts in the medical and psychiatric literature before the last quarter of the eighteenth century. It is impossible to read of the private Rivet clinic without feeling that, in the world of psychiatric disease, a new entity had raised its head.

Dead End

By 1900, psychiatry had reached a dead end. Its practitioners were concentrated for the most part in asylums, and asylums had become mainly warehouses in which any hope of therapy was illusory. Psychiatrists themselves had a rather poor reputation among their medical colleagues as the dull and the second-rate, just a step, if that, above the spa-doctors and the homeopaths.

Bulging with chronic paretics, dements, and catatonic schizophrenics, the asylums had achieved a desolateness that would have made the earlier generation of reformers heartsick. William Alanson White, superintendent of St. Elizabeths Hospital in Washington, had trained at the Blackwell’s Island asylum in New York. He recalled rowing over early one morning from Manhattan and landing at a pier close to the asylum. “I could look back a distance of a few hundred feet and see the building where the women patients were confined. My memory of that vision is a building in which each window was a glare of light, and I can hear now the sounds that issued therefrom which reminded me of a hive of bees. These disturbed women, as they were called, were obviously in a state of continuous noisy activity all night long....” What could one do about those who were too noisy? White described the restraining sheets used in the 1890s at the Binghampton State Hospital on violent patients in the absence of other (then taboo) mechanical

restraints. “A perfectly hellish contrivance, literally speaking, in hot weather; and I am sure I have seen at least one patient die from heat exhaustion as a result of it.”[118] Such an institution, of course, would be awful for the patients, but it also would be demoralizing for the physicians who served there. A profession composed of practitioners who did this kind of thing for a living would not hold its head up.

Britain was no different. In the judgment of one elderly psychiatrist formerly at the Buckinghamshire County Pauper Lunatic Asylum, “Between 1860 and 1930, say, British asylums remained backwaters.”[119] The alienists themselves lost much of their contact with medicine, the life of a superintendent resembling more that of a gentleman farmer, “invited to tennis parties at the Rectory, fishing and even riding to hounds,” as Eliot Slater, staff psychiatrist in the 1930s at the Maudsley Hospital in London, later said.

“Life was not quite so pleasant for the patients,” Slater continued. “Confined to locked wards with an enclosed court for exercise, patients whose stormy illness had blown itself out leaving only residual symptoms, would remain year after year, unoccupied, deprived of incentive or responsibility of self-determination, getting more and more fixed in the straitjacket of an unchanging daily routine.”[120]

From the 1830s on, the British had prided themselves greatly on abolishing mechanical restraints. Yet in an exercise that Montagu Lomax called “straining out a gnat and swallowing a camel,” the British relied on (supposedly illegal) isolation cells that humiliated the patients even more than tying them down. One evening Lomax was called back to the asylum because a paralytic patient had fallen out of bed and broken his leg. “As I went through one of the wards on my way to the hospital I passed a single room in which a refractory patient was ‘secluded.’ It was after 7 P.M., and no attendant was within call. The patient was beating on the door with his fists and feet, and was shrieking out curses and imprecations. “For God’s sake let me out, doctor! for God’s sake let me out! O Christ, they are killing me! for God’s sake let me out!”

“As I came back, after setting the broken leg, the same horrible sounds greeted my ears. They would probably continue for hours, keeping everyone in the vicinity awake, and ultimately might necessitate my giving the man a hypodermic injection. My reflections were not pleasant.”[121]

Toward 1900, German asylums were probably the best-run in the world, because the German states supported them liberally, and because asylum

physicians in Central Europe still retained the drive to do scientific research, a talisman of better care. (Care was also boosted by the "Herr Geheimrat" phenomenon, physicians intent on honors, who wanted to demonstrate themselves to the Ministry as medically up to snuff: This was almost unknown elsewhere.) Yet even in this birthland of reform, psychiatric conditions were upsetting. As Emil Kraepelin, later the most prominent of the academic psychiatrists, arrived at his new job as assistant physician at the Munich City Asylum in 1878, he had a migraine. A Mecklenburger from the North, he didn't know Bavaria at all. And the superintendent had put him in charge of the men's back ward. Kraepelin's thoughts were not pleasant as he confronted these 150 "demented, unclean [fecal is meant], half-agitated and fully agitated patients," many of whom were incapable of work and would hang around the hallways and courtyards, "where they would run around, yell, get into fights with each other, collect rocks, smoke, and chatter. The tendency to violence was very widespread; there was scarcely a daily rounds in which someone didn't report a fight, window breaking or destroying tableware. Often enough I had to suture or bind up the wounds that arose in this way."

Most fearsome in Kraepelin's daily itinerary was Ward G, housing the most violent of the male patients. One patient, locked in an isolation room, had managed to shatter a fellow patient's skull with a scrub brush. Later, the same patient nearly throttled an incautious attendant, took his keys away, and escaped into the city. As the patient was on the point of hurling a passerby into the Isar, the river that runs through Munich, he was overpowered and returned to the asylum. "Nobody," said Kraepelin, "had noticed he was missing."[122] This was not what Jacobi had in mind.

Not only had asylums themselves sunk to a historic low point, the profession of psychiatry had as well. "Who becomes a psychiatrist?" asked German psychiatrist Werner Heinz in 1928, tongue in cheek: (1) "Applicants for county medical officer who are afraid of failing the special exam in psychiatry if they don't get some practical experience"; (2) Those who are "physically inadequate," having rheumatism or a heart problem, or who "otherwise are not up to the exertions of a rural practice, maybe not even an urban practice"; (3) Those who are "intellectually inadequate, who instinctually seek out an asylum because they will stand out less there, and it's these latter, believe me, who later become the *superintendents*."[123]

Early in 1908, the superintendent of the Rheinau asylum in Switzerland

asked the young Dr. Karl Gehry to consider a post there. Gehry was torn. On the one hand, he could face the uncertainties of opening a private village practice. On the other hand, “There was abroad a deep mistrust of psychiatry.” His teacher, the famous Carl Jung, had told him in Zurich that “psychiatry is a stepchild of medicine” because it is not classed among the natural sciences, not weighing or measuring anything. “And people simply didn’t think that a psychiatrist could do much. He simply declared everyone to be more or less mentally ill.” Nobody, continued Gehry, would find it striking if he became a surgeon, “because surgery could support a family and had status. Psychiatry banishes you to an asylum where you’re mainly isolated. The psychiatrist has to treat people who don’t think they need treatment so that every day he has to swallow all those insults....”[124] Such, roughly, was the position of everyday psychiatry.

Thus by 1900, the status of psychiatrists themselves had reached a low point. What a contrast with the earlier arrogant refusal of psychiatrists to associate themselves with the boil-lancers and enema-givers who constituted the rest of medicine. In 1853, the newly founded American organization of asylum physicians had refused to associate with the American Medical Association.[125] But then this self-imposed isolation had started to chafe. Alienists squirmed as Philadelphia neurologist and society nerve-doctor Weir Mitchell, in 1894, delivered his historic rebuke: “I shall have frankly to reproach ... many of those who still bear the absurd label of ‘medical superintendents,’” he told them at their annual meeting. “Where are your annual reports of scientific study, of the psychology and pathology of your patients?” he asked them. “We commonly get as your contributions to science, odd little statements, reports of a case or two, a few useless pages of isolated post-mortem records, and these are sandwiched among incomprehensible statistics and farm balance-sheets.” Treatment? A sham, declared Mitchell: “Whatever the gullible public might believe about therapy, we [neurologists] hold the reverse opinion, and think your hospitals are never to be used save as the last resource.”[126] Few psychiatrists could have emerged from this meeting without asking themselves, Am I still a physician?

Scarcely, answered William Bullard, president of the American Neurological Association at its meeting in Boston in 1912. To be sure, psychiatrists had done wonderfully, he sneered, at “direct[ing] the building and heating of their buildings, the buying of coal and groceries, the making

up of accounts even to minute details." The problem was that in so doing they had lost contact with the rest of medicine.[127] In 1933, White declared most asylum physicians to be simply "dead wood."[128]

"Anchored in the setting of chronic illness, distant from the mainstream of medicine," as one rueful psychiatrist put it many years later, asylum psychiatry had come to the end of the road.[129] To establish that there was such a thing as a science of the brain and mind, and that the use of this science could help make patients better, the torch had to pass to other hands.